

CATEGORY 2 - FULL PROJECT PROPOSAL

FairBanks Community Health Reach Project

An Integrated Community-Based Model for Preventive Healthcare, Early Detection, Referral and Continuity of Care in Underserved Communities of Kampala, Uganda

Proposed Duration: October 2026 - September 2027

Proposed Budget: USD 49,500

Submission Deadline: 2 September 2026

Submitted to: Within Africa Foundation

Item	Details
Applicant Organization	FairBanks Medical Centre Limited
Project Title	FairBanks Community Health Reach
Project Focus	An Integrated Community-Based Model for Preventive Healthcare, Early Detection, Referral and Continuity of Care
Country	Uganda
Project Location	Kampala, with an initial focus on Kyebando-Kisalosalo and surrounding underserved peri-urban communities.
Project Duration	12 months
Proposed Implementation Period	October 2026 - September 2027, subject to award confirmation
Total Project Budget	USD 49,500
Funding Requested from WA Foundation	USD 49,500
Project Contact Person	Racheal Nabukeera, Founder & Managing Director

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Acronyms

AAR	-	African Air Rescue
CHEW	-	Community Health Education Worker
CHW	-	Community Health Worker
CIC	-	Co-operative Insurance Company
FCHIP	-	Fairbanks Community Health Intelligence Programme
FCHR	-	Fairbanks Community Health Reach
IT	-	Information Technology
KCCA	-	Kampala Capital City Authority
LC	-	Local Council
M&E	-	Monitoring & Evaluation
USD	-	United States Dollars
VHT	-	Village Health Teams
WA	-	Within Africa

1 EXECUTIVE SUMMARY

FairBanks Community Health Reach (FCHR) is an integrated community-based model for preventive healthcare, early detection, referral and continuity of care among underserved peri-urban communities in Kampala, Uganda.

The project responds to a practical set of barriers that can prevent people from obtaining timely and appropriate healthcare. These include; limited access to preventive health information, delayed health-seeking behavior due to financial constraints & cultural beliefs, weak or fragmented referral pathways and loss of continuity after a patient encounters the formal healthcare system. The result can be delayed identification of health risks and missed opportunities for early intervention.

FairBanks Community Health Reach will establish a structured bridge between community-level health promotion and formal clinical care. The model will work through existing community structures, including Village Health Teams (VHTs), Community Health Workers (CHWs), local leaders, youth leaders, women’s group leaders and religious leaders, linking them to FairBanks Medical Centre as a clinical and coordination hub.

The project will operate through a community engagement approach and cover the 08 cells in Kyebando Parish (Central A, Erisa, Kalerwe, Quarters, Katale, Kitabuliiki, Nsooba and Tuula) and the cells in the neighbouring parishes, engage and orient at least 30 VHTs, CHWs, reach out to at least 6,000 community members through targeted community engagements and health education activities, screen at least 2,000 individuals, initiate and track at least 300 referrals, achieve at least 70% completion of trackable referrals, document at least 500 follow-up contacts for priority cases through the FairBanks Community Health Intelligence Platform (FCHIP), develop and implement a sustainability-partnership plan.

Implementation will take place in four broad phases: (i) Inception and Baseline to establish the project foundation, map and understand the target communities and determine the starting situation; (ii) Community Rollout to introduce Fairbanks Community Health Reach project to the target communities, delivery health service, carry out mobilization, health education and screening activities; (iii) Integrated Care and Referral strengthening that connects community-level identification and screening to appropriate clinical care through structured referral tracking and patient follow-up using the Fairbanks Community Health Intelligence Platform (FCHIP); (iv) Consolidation, Learning and Sustainability through assessing results, documenting lessons & outcomes, strengthening partnerships and establishing mechanisms for continuity such the Community Health Insurance initiative. These activities will be financed by the estimated project budget is USD 49,500.

FCHR is designed as a catalytic intervention rather than a short-term outreach campaign. By the end of the project, FairBanks expects to have strengthened community relationships, referral mechanisms, follow-up systems, health information processes, partnerships and sustainability pathways that can support continuation and future scale-up beyond the grant period.

2 PROJECT JUSTIFICATION

2.1 Rationale

FairBanks Medical Centre Limited was established in 2025 as a private healthcare facility, initially serving patients through cash payments, health insurance and corporate arrangements. As FairBanks reviewed its first year of operation, it observed that some residents from surrounding communities needed healthcare but could not afford the cost of the services they required, prompting management to reflect on how the Centre could use its existing healthcare capacity to respond more meaningfully to the needs of the population around it.

This reflection led to a gradual change in the facility's direction and began moving beyond a model focused mainly on fee-paying patients towards a social-enterprise approach to healthcare, which birthed FairBanks Community Health Reach (FCHR) project. The concept is built around connecting communities with the health facility through community engagement, health education, screening, referral, clinical linkage, follow-up and the use of community health information. This reflects the facility's transformation described in the original concept note, which places community health, prevention, partnerships, digital health and sustainable financing alongside facility-based clinical care.

The rationale for FCHR is therefore to create a structured bridge between the community and healthcare services. The VHTs, CHWs and other trusted community structures can help people access health information, identify potential risks and initiate appropriate referrals, while FairBanks provides the clinical linkage, follow-up and continuity of care. At the same time, the FairBanks Community Health Intelligence Platform (FCHIP) can help capture information on community reach, screening, referrals and follow-up so that the project can identify gaps and improve implementation.

The project is consequently not intended to be a stand-alone medical outreach campaign, It is intended to establish a more connected and sustainable way of working, in which community engagement, prevention, early detection, referral, treatment and follow-up reinforce one another.

2.2 Problem Statement

For many people living in underserved communities, getting the right healthcare at the right time is still a challenge. Health information on prevention, healthy lifestyles, maternal and family health, chronic diseases and when to seek care does not always reach people in a clear and consistent way. As a result, some health problems may only be discovered when they have already become more serious. Even when a concern is identified, getting from the community to the right health service can be difficult. A referral may be made, but the person may not reach the facility, secure an appointment or complete the recommended care. After treatment, some patients may also struggle to continue with follow-up, medication or other support they need to stay well. At the same time, health teams often have limited information about what is happening between the community and the health facility, making it harder to see where patients are being lost along the way and where the system needs to improve. FairBanks Community Health Reach is designed to respond to these realities by creating a practical link between

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communities and healthcare services; helping people access health information, identify risks earlier, reach appropriate care and receive the follow-up they need.

2.3 Why the Project Is Needed

FCHR is needed because effective community healthcare requires more than one-off health campaigns. A sustainable pathway should allow community members to move from health information and prevention, to early identification, to appropriate referral, to clinical care, and then to follow-up and continuity. The FCHR model seeks to establish this pathway within different communities while generating practical evidence on what works, where referral bottlenecks occur, how communities participate and which approaches are most suitable for continuation or scale-up.

2.4 Project Alignment with FairBank’s Vision and Mission

FCHR advances FairBanks Medical Centre’s broader direction of providing accessible, compassionate and sustainable healthcare while connecting clinical services with community health, innovation, partnerships and sustainable financing. The project positions FairBanks Medical Centre not simply as a place where patients receive treatment, but as a community-linked health coordination hub that supports prevention, early detection, referral, clinical linkage, follow-up and learning.

2.5 Key Interventions and approaches to be implemented

Intervention	Approach	Intended Value
Community mobilisation	Map priority communities and engage VHTs, CHWs, local leaders, youth leaders, women’s groups and religious leaders.	Establish a functioning community entry, participation and referral network.
Health education	Conduct structured sessions on prevention, healthy lifestyles, maternal/family health, chronic disease awareness and appropriate health-seeking.	Improve access to health information and support preventive behaviour.
Screening and early detection	Conduct priority community screening and risk identification, followed by counselling and referral where indicated.	Identify individuals who may require timely clinical assessment.
Referral and clinical linkage	Establish structured community-to-facility referrals, appointment linkage and coordination with appropriate clinical services.	Improve access and referral completion.
Follow-up and continuity	Conduct post-visit follow-up, adherence support and selected home/community follow-up for priority cases.	Reduce loss to follow-up and strengthen continuity of care.
FCHIP and health intelligence	Capture community information, track referrals and generate routine programme dashboards.	Strengthen evidence-informed programme management and decision-making.
Sustainability and partnerships	Establish community insurance schemes, community financing, partnership and scale-up pathways.	Increase the likelihood of continuation beyond the grant period.

Table 1: Intervention Matrix

2.6 Community Engagement Strategy

Community participation is a central implementation mechanism of FCHR. The engagement will begin with stakeholder mapping and a structured meeting process, then move into mobilisation, sensitisation, household identification, referral support and community feedback.

Stakeholder Group	Key Engagement / Contribution	Expected Output
LCI and II Chairpersons	Community mobilisation; support access and permission to hold sessions; local coordination.	Community entry secured; local mobilisation supported; agreed responsibilities.
VHTs	Community mobilisation; household identification; record keeping of sessions, households reached and referrals; referral support and follow-up.	Community households identified; referrals documented; activity records maintained.
Youth Leaders	Mobilisation of members especially the youth; sensitisation talks.	Youth participation and sensitisation activities documented.
Women Group Leaders	Mobilisation of members especially women; sensitisation talks.	Women’s group participation and sensitisation activities documented.
Religious Leaders	Public announcements and outreach activities in places of worship.	Faith-based communication channels activated.

Table 2: Community Engagement and Expected Output

The stakeholder meetings will: (i) share the community engagement plan; (ii) communicate FairBanks expectations; (iii) identify stakeholder expectations of FairBanks Medical Centre; and (iv) agree on the best way forward and responsibilities.

3 PROJECT GOAL

To improve health equity and well-being among underserved peri-urban communities in Kampala by strengthening a sustainable community-to-facility healthcare system that enables people, particularly those facing financial, age-related and other barriers, to access preventive care earlier, receive appropriate treatment and remain connected to essential health services throughout their care journey.

4 PROJECT ACTIVITIES AND DELIVERABLES

The FairBanks Community Health Reach project will be implemented through nine interconnected work packages. The community engagement component is embedded within the inception, mobilisation, education, referral and monitoring work packages so that stakeholder participation is part of the core project delivery pathway rather than a parallel activity.

FCHR PROJECT ACTIVITY SCHEDULE —OCTOBER 2026 – SEPTEMBER 2027															
Sl	Phase	Activity	Q1 (Oct–Dec 2026)			Q2 (Jan–Mar 2027)			Q3 (Apr–Jun 2027)			Q4 (Jul–Sept 2027)			Deliverables
			Oct	Nov	Dec	Jan	Feb	Mar	Apr	May	June	Jul	Aug	Sept	
1	Inception and Baseline	Project Management and Administration													Management plan, Budget, meeting minutes, progress/performance reports, issue log, action tracker, expenditure reports, supplier records, PMC reports, decision logs, final project report.
		Stakeholder mapping and database development.													Stakeholder map; verified stakeholder database; amme tools and referral protocols.
		Meetings with local leaders, VHTs, youth leaders, women group leaders and religious leaders to share the engagement plan, clarify FairBanks expectations, hear stakeholder expectations and agree the way forward.													Meeting minutes; stakeholder expectations/action matrix.
		Develop tools, strategies and approaches for implementation													Tools, community map; baseline summary and Agreed-on Activity schedule.
		Quarterly Financial and technical reporting to funders.													Quarterly technical report, quarterly financial report, updated workplan, KPI framework, procument/contract status.
2	Community Rollout	Orient VHTs/CHWs on mobilisation, household identification, health education, basic screening support, referral, follow-up and record keeping.													Oriented VHT/CHW network; supervision records.
		Community mobilisation													Mobilisation plans; household identification records; mobilisation records.
		Support local leader participation in mobilisation and access to community sessions.													Mobilisation plans; household identification records; mobilisation records.
		Establish supervision and communication mechanisms.													documented key messages.
		Deliver structured community sensitisation and health education sessions.													At least 48 structured sessions; attendance registers.
		Mobilise youth and women groups for sensitisation talks.													Mobilisation records.
		Use religious leaders for public announcements/outreach.													Community actor support records.
		Conduct priority community screening and risk identification.													Screening records; risk-identification summary.
3	Intergrated Care and Referral Strengthening	Provide counselling and referral where indicated.													Referrals.
		Maintain a community-to-facility referral register													Referral register.
		Support appointment linkage and navigation.													Clinical linkage records.
		Coordinate assessment and treatment at FairBanks or appropriate referral services.													Clinical linkage records; referral review summary.
		Track referral completion and reasons for non-completion.													Priority case tracking; continuity-of-care reports; at least 500 documented follow-up contacts.
		Conduct post-visit follow-up for priority cases.													Follow-up records.
		Provide appropriate adherence and care-navigation support.													Referral completion tracking; appointment linkage records.
		Conduct selected home/community follow-up.													Referral/follow-up tracking system.
		Escalate cases requiring additional action.													Clinical linkage records; referral review summary; priority case tracking; continuity-of-care reports.
		Capture community information and participation data.													Updated FCHIP tools.
4	Consolidation, Learning and sustainability	Track referrals and follow-up.													Follow-up records.
		Generate basic dashboards and routine programme intelligence.													Routine dashboards; programme intelligence reports.
		Use data in management reviews and adaptive action.													
		Validate baseline and targets.													Baseline validation.
		Conduct routine monitoring and data quality checks.													Routine monitoring records.
		Hold quarterly programme reviews.													Quarterly review reports.
		Collect community/client feedback.													Client feedback register.
		Conduct endline assessment and document lessons.													Endline assessment; lessons-learned report.
		Develop insurance/community financing pathways where feasible.													Sustainability plan.
		Engage government, private-sector, development and community partners.													Partnership pipeline; engagement records.
		Explore co-financing and resource-leveraging opportunities.													Financing pathway recommendations.
		Develop continuation and scale-up options.													Scale-up recommendations.
		Monthly activity review and workplan tracking.													Monthly management reviews.
		Quarterly Financial documentation and expenditure monitoring.													Quarterly Financial records.
		Community feedback and complaints/concerns escalation.													Community feedback log.
		Funder narrative and financial reporting.													Donor reports; evidence file ready for verification.

Figure 1: Activity Schedule

5 PROJECT RESULTS

The project will enable underserved peri-urban communities in Kampala to access timely preventive healthcare, identify health risks earlier, complete referrals more effectively, remain connected to care, and strengthen the use of community health information to support better health outcomes and sustainable service delivery as detailed in the table 3.

Expected Output	Indicator	Target	Expected Outcome
Priority communities mapped and engaged through a documented stakeholder process.	Priority communities mapped and engaged	At least 08	Improved access to preventive health information and priority screening among targeted communities.
VHTs, CHWs and relevant community actors oriented and supported to participate in the programme model.	VHTs/CHWs oriented	At least 30	Strengthened community-based health workforce capacity and participation.
Community members reached through health education, mobilisation and sensitisation.	Community members reached through education/mobilisation	At least 6,000	Increased community awareness, knowledge and adoption of appropriate health-seeking and preventive health practices, with improved utilisation of available health services.
Priority screening activities conducted and at-risk individuals identified for appropriate follow-up or referral.	Individuals screened	At least 2,000	Earlier identification of priority health risks and conditions requiring clinical assessment.
Community-to-facility referrals initiated, documented and tracked.	Individuals identified for further assessment/referral	At least 300	Improved referral completion and linkage between community structures and appropriate clinical services.
Priority cases supported through follow-up and continuity mechanisms.	Referrals initiated and tracked	At least 300	Improved continuity of care through documented follow-up.

Community health and referral information captured through FCHIP.	Completion of trackable referrals	To be Established during implementation	Stronger use of community health information for programme planning and service improvement.
Quarterly programme reviews and learning actions completed.	Documented follow-up contacts	At least 500	Improved programme performance, accountability and effectiveness through evidence-based learning, timely adaptation and continuous improvement.
Partnership and sustainability pathways developed for continuation beyond the grant period.	Partnership and sustainability pathways developed	At least 48	Greater readiness for continuation and scale-up through partnerships, financing pathways and institutional leverage.
	Programme reviews	Quarterly, plus endline assessment	Greater readiness for continuation and scale-up through partnerships, financing pathways and institutional leverage.

Table 3: Project's Expected Output and Outcome

6 PROJECT IMPLEMENTATION PLAN

Implementation will be led by FairBanks Medical Centre through a phased 12-month approach. The project will use the Community Engagement Plan as the operational entry mechanism for community participation. Its 12-week stakeholder-to-community mobilisation sequence is embedded primarily within project Months 1-3, while referral, follow-up, data capture and stakeholder review continue across later phases.

Phase	Months	Key Actions	Main Outputs
Phase 1 - Inception and Stakeholder Foundation	M1-M2	- Stakeholder mapping - Stakeholder database - Introductions - First engagement meeting - Expectations assessment - Agree responsibilities - Community mapping	- Inception package - Stakeholder - Map/database - Meeting records - Baseline - Oriented network - Operational tools

		• Baseline • VHT/CHW orientation • Tools • Referral protocols	
Phase 2 - Community Rollout	M3-M7	• Community mobilisation • Sensitisation • Youth/women group outreach • Religious announcements • Household identification • Screening • Referral initiation • Initial follow-up	• First full outreach cycle • Screening and referral pathway operating • Community records
Phase 3 - Integrated Care and Referral Strengthening	M3-M7	• Routine outreach • Referral strengthening • Clinical linkage • Follow-up • FCHIP dashboards • Monthly stakeholder review • Quarterly learning review • Partnership development	• Regular community-to-facility pathway • Referral tracking • Dashboards • Review actions • Partnership pipeline
Phase 4 - Consolidation and Sustainability	M10-M12	• Outcome/endline assessment • Learning • Second/periodic stakeholder review • Sustainability and financing planning • Partnership consolidation • Scale-up recommendations	• Endline report • Sustainability plan • Documented partner actions • Scale-up recommendations

Table 4: Project Implementation Plan

6.1 Community Engagement Sequence Embedded in the Implementation Plan

Engagement Step	Embedded Project Timing	Responsible Lead	Expected Evidence
Stakeholder mapping	M1	Community Engagement Team	Stakeholder map/list
Stakeholder database	M1-M2	Community Engagement Team	Verified contact database
Introductions/contact	M2	Management + Engagement Team	Contact log
Stakeholder meetings	M2	Management + Stakeholders	Attendance and minutes
Expectations assessment	M2	Management + Stakeholders	Expectations matrix
Agree responsibilities/way forward	M2	Management + Stakeholders	Action plan/responsibility matrix
Community mobilisation	M3 onward	Engagement Team + Leaders	Mobilisation records
Sensitisation sessions	M3 onward	FairBanks Team + Leaders	Session registers
Household identification	M3 onward	VHTs + CHWs	Household records
Referral support	M3 onward	VHTs + CHWs + FairBanks	Referral register
Religious outreach	M3 onward	Religious Leaders + FairBanks	Announcement/outreach log
Activity documentation	Throughout	VHTs + CHWs + Engagement Team	Updated records
Stakeholder review	Quarterly / as scheduled	Management + Stakeholders	Review minutes and action log

Table 5: Community Engagement Sequence Matrix

7 PROJECT SCOPE AND WORK BREAKDOWN STRUCTURE

The project scope covers the community-facing and community-to-facility activities required to establish and operate the FairBanks Community Health Reach model during the 12-month period. The Community Engagement Plan is treated as a core delivery mechanism within this scope. Routine institutional overheads are not presented as the primary funding request.

WBS Code	Work Item
1.0	Project Management, Inception and Stakeholder Engagement
1.1	Stakeholder mapping and database development
1.2	Stakeholder introductions and first engagement meeting
1.3	Stakeholder expectations assessment and responsibility agreement

1.4	Community mapping and baseline
1.5	Programme tools, safeguarding and referral protocols
2.0	Community Mobilisation and VHT/CHW Support
2.1	VHT/CHW orientation
2.2	Local leader engagement and mobilisation
2.3	Youth and women group mobilisation
2.4	Religious outreach and announcements
2.5	Household identification
3.0	Health Education, Sensitisation and Screening
3.1	Community education/sensitisation sessions
3.2	Priority screening
3.3	Risk identification and counselling
4.0	Referral and Clinical Linkage
4.1	Referral initiation
4.2	Appointment linkage/navigation
4.3	Clinical assessment/treatment linkage
4.4	Referral completion tracking
5.0	Follow-up and Continuity
5.1	Post-visit follow-up
5.2	Selected home/community follow-up
5.3	Adherence and care-navigation support
6.0	FCHIP, Data and Programme Intelligence
6.1	Community information capture
6.2	Referral/follow-up tracking
6.3	Dashboards and routine programme intelligence
6.4	Data quality checks
7.0	Monitoring, Evaluation and Learning
7.1	Baseline validation
7.2	Routine monitoring
7.3	Quarterly reviews
7.4	Community/client feedback
7.5	Endline assessment and learning
8.0	Sustainability and Partnerships
8.1	Insurance/community financing pathways

8.2	Strategic partnership development
8.3	Co-financing/resource leverage
8.4	Scale-up and post-grant continuity planning
9.0	Accountability and Reporting
9.1	Financial controls and documentation
9.2	Community accountability and complaints/feedback
9.3	Funder narrative and financial reporting

Table 6: Work Breakdown Structure

7.1 Scope Boundaries and Deliverables

Project Interventions	Primary Evidence / Deliverable
Community-facing engagement and mobilisation	Stakeholder records, mobilisation reports, session registers
VHT/CHW participation and support	Orientation records, supervision records
Screening and referral	Screening records, referral register, completion reports
Follow-up and continuity	Follow-up records, continuity report
FCHIP/data systems	Data tools, dashboards, programme intelligence
M&E and learning	Baseline, quarterly reviews, endline
Sustainability and partnerships	Sustainability plan, partner pipeline, scale-up plan
Routine institutional overheads unrelated to project delivery	Not the primary grant request

Table 7: Primary Deliverable Matrix

8 PROJECT RESOURCES AND CONTRIBUTIONS

The project will combine FairBanks institutional contributions, community participation and catalytic grant financing. The funding request is designed to support community-facing activities and measurable outputs, while FairBanks will contribute existing infrastructure, management capacity, clinical coordination, administrative systems and other institutional resources as appropriate.

Resource Area	FairBanks Contribution	Community Contribution	WA Foundation Contribution
Facility and clinical platform	Existing medical centre, clinical coordination and routine institutional infrastructure.	Use of community access points and community mobilisation structures.	Costs for catalytic project-linked service inputs and community programme costs.
Human Resources	Management, clinical and administrative oversight; existing staff capacity as appropriate.	VHT/CHW participation and local leadership engagement.	Funds for training/orientation, mobilisation support and programme coordination costs.
Data and digital systems	Institutional FCHIP concept and existing digital/administrative capacity.	Participation in data collection and feedback.	Funds for tools, data capture, referral tracking, dashboards and programme intelligence.

Partnerships	Existing partnership-development capacity and institutional coordination.	Community organisations/local structures as available.	Partnership development and sustainability-building activities.
Co-financing / leverage	Institutional contribution through existing infrastructure, staff capacity and systems.	Time, community mobilisation and local participation where appropriate.	USD 49,500 project financing

Table 8: Project Resources & Contribution Matrix

9 PROJECT BUDGET

The proposed Category 2 budget is USD 49,500. The allocation follows the project structure already developed for the concept submission and is intended to finance catalytic programme activities and measurable outputs. Final unit costs should be validated against competitive quotations and any final WA Foundation budget requirements before contracting.

Phase	Activity	Deliverable	Justification	Qty	Unit	Unit Cost (USD)	Total Cost (USD)
INCEPTION	Project management and administration	Project management and administrative systems established	Project coordination, work planning, coordination meetings, documentation, communication and administrative support	12	months	679	8,148
	Stakeholder mapping and database development	Stakeholder map and verified database	Staff time, field transport, communication, data collection tools, data entry and database development	8	communities	150	1,200
	Develop project tools, strategies and implementation approaches	Project tools, strategies and implementation approaches finalised	Technical staff time, consultations, tool development, printing and implementation/referral tools	1	package	1,300	1,300
	SUBTOTAL						10,648
BASELINE	Baseline meetings and consultations	Baseline consultation records and validated findings	Meeting space, refreshments, transport, facilitation, stationery, communication and documentation	8	meetings	125	1,000
	Baseline assessment/data collection	Baseline data and assessment report	Data collection tools, field personnel, transport, data entry, analysis and reporting	1	assessment	1,000	1,000
	Quarterly financial and technical reporting to funder	Quarterly financial and technical reports submitted	Financial reconciliation, data compilation, KPI updates, report writing,	4	reports	125	500

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			documentation and submission				
	SUBTOTAL						2,500
COMMUNITY ROLLOUT	VHT/CHW orientation	Oriented VHT/CHW network	Training venue, materials, facilitator, meals/refreshments, transport and stationery	30	VHTs/CHWs	67	2,000
	Community mobilisation	Communities mobilised and households identified	VHT/CHW mobilisation, transport, airtime, mobilisation materials and household identification tools	12	communities	150	1,800
	Local leader engagement	Local leaders engaged to support implementation	Meetings, communication, transport, sensitisation materials and documentation	12	engagements	50	600
	Community sensitisation and health education	Community members reached through health education and sensitisation	Facilitators, transport, materials, community venue, communication and documentation	25	sessions	80	2,000
	Youth group sensitisation	Youth groups reached with priority health information	Facilitator, mobilisation, transport, communication and IEC materials	14	sessions	50	700
	Women's group sensitisation	Women's groups reached with priority health information	Facilitator, mobilisation, transport, communication and IEC materials	14	sessions	50	700
	Religious leader outreach	Religious/community networks engaged in health promotion	Religious leader engagement, communication, outreach materials and transport	10	engagements	50	500
	Priority community screening	Individuals screened and priority health risks identified	Screening personnel, equipment, consumables, PPE, forms, transport, data capture and clinical supervision	1,500	individuals	2	3,000
	Community health risk identification	Priority individuals identified for further assessment/support	Screening tools, VHT/CHW support, clinical review and documentation	200	individuals	2	400
	SUBTOTAL						11,700

INTEGRATED CARE & REFERRAL STRENGTHENING	Counselling and referral	Individuals requiring further care appropriately referred	Counselling, referral forms, communication, referral guidance and documentation	300	referrals	3	900
	Community-to-facility referral system	Functional referral pathway and referral records	Referral register, forms, data entry, communication and coordination	12	months	50	600
	Appointment linkage and navigation	Referred clients supported to access appropriate services	Telephone/SMS/Whats App communication, appointment coordination and navigation	300	referrals	4	1200
	Referral tracking	Referral completion and non-completion documented	Follow-up calls, VHT/CHW tracking, data entry and referral review	300	referrals	3	900
	Clinical linkage	Referred clients connected to appropriate clinical assessment/care	Clinical coordination, appointment support, referral communication and documentation	300	clients	8	2400
	Patient follow-up	Follow-up records for priority cases	Telephone calls, VHT/CHW follow-up, communication and documentation	500	contacts	3	1500
	Home/community follow-up	Priority cases followed up within the community	VHT/CHW time, transport, communication and follow-up tools	100	visits	20	2000
	Escalation of priority cases	Cases requiring additional intervention appropriately escalated	Clinical review, communication, referral coordination and documentation	50	cases	12	600
	SUBTOTAL						10,100
FCHIP	Community information and participation data capture	Community participation and health activity data captured	Digital data collection tools, devices, internet/data, data entry and staff time	12	months	100	1,200
	Referral and follow-up tracking	Referral and follow-up information captured and updated	Digital tracking, data management, internet/data and technical support	12	months	70	840
	Dashboard development and updating	Routine project dashboards generated	Dashboard development, data analysis, software/platform support and reporting	4	dashboards	200	800

	Use of project data for management decisions	Data-informed management actions documented	Data analysis, management review meetings, action tracking and documentation	4	reviews	175	700
	SUBTOTAL						3,540
M&E	Routine project monitoring	Routine monitoring records and progress information available	Monitoring tools, field visits, transport, data collection and staff time	12	months	100	1,200
	Data quality checks	Verified and reliable project data	Data verification, field checks, database review and correction	4	checks	100	400
	Quarterly project reviews	Quarterly review reports and action plans	Meeting costs, data analysis, refreshments, documentation and action planning	4	reviews	250	1,000
	Community/client feedback	Community/client feedback documented and addressed	Feedback tools, community discussions, communication, data capture and analysis	4	rounds	100	400
M&E	Endline assessment	Endline assessment and comparison against baseline	Data collection, field personnel, transport, data analysis, validation and reporting	1	assessment	1,500	1,500
	Lessons learned documentation	Lessons learned and implementation recommendations documented	Data analysis, stakeholder consultations and report preparation	1	report	500	500
	SUBTOTAL						5,000
SUSTAINABILITY & PARTNERSHIPS	Develop insurance/community financing pathways	Community financing pathway recommendations	Research, stakeholder consultations, technical input, meetings and documentation	1	package	600	600
	Government and partner engagement	Government, private-sector and development partners engaged	Meetings, transport, communication, presentation materials and documentation	12	engagements	75	900
	Explore co-financing/resource-leveraging opportunities	Potential co-financing/resource opportunities identified	Partner meetings, communication, concept development and documentation	6	opportunities	100	600
	Develop continuation and scale-up options	Sustainability and scale-up plan	Data analysis, stakeholder	1	plan	900	900

			consultations, technical input and planning				
	SUBTOTAL						3,000
PROJECT MANAGEMENT & ACCOUNTABILITY	Financial documentation and expenditure monitoring	Complete and up-to-date financial records	Bookkeeping, expenditure reconciliation, budget tracking and supporting documentation	4	quarters	250	1,000
	Community feedback and complaints/ concerns escalation	Feedback and complaints appropriately recorded and addressed	Feedback register, communication, investigation/escalation and documentation	12	months	42	504
	Procurement and supplier administration	Procurement and supplier records maintained	Quotations, procurement documentation, supplier records and contract administration	12	months	42	504
	Project accountability and financial close-out	Complete project financial and administrative records	Financial review, reconciliation, close-out documentation and records management	1	project	1,000	1,000
	SUBTOTAL						3,008
					TOTAL PROJECT BUDGET		49,496

Table 9: Project Budget Matrix

9.1 Budget Controls

Grant funds will be managed through a defined approval process designed to provide clear checks and balances at every stage of expenditure. No project expenditure will be incurred without prior approval by the designated responsible officers and verification that the expenditure is included in the approved project budget.

All requests for expenditure will originate from the responsible project function and will be supported by appropriate documentation, including requisitions, quotations, specifications or other relevant supporting records. The request will then pass through the designated review and approval levels before procurement or payment is authorised. Where applicable, procurement and financial reviews will confirm the availability of funds, budget line, value for money and compliance with approved procedures.

The project will maintain an auditable record of approvals, procurement documents, invoices, payment records and expenditure reports. This approval system will enable FairBanks Medical Centre Limited to ensure that grant resources are used only for authorised project purposes, by accountable personnel, and in accordance with the approved budget and donor requirements.

Control Area	Project Assumption / Procedure
Procurement basis	Procure project goods and services only where they are directly linked to approved activities and budget lines.
Cost validation	Validate proposed unit costs through competitive quotations, market checks or other documented price evidence before commitment.
Supplier selection	Select suppliers/service providers using documented, value-for-money considerations and appropriate authorisation.
Budget control	Commitments and expenditure must remain within approved budget lines unless formally authorised through the applicable change process.
Documentation	Maintain quotations, purchase requests, approvals, invoices, delivery/receipt evidence and payment support as applicable.
Segregation of duties	Separate requesting, approval, purchasing/receiving and payment functions to the extent practicable within the project structure.
Receipt and verification	Confirm that goods/services have been received and are fit for purpose before payment is processed.
Conflict of interest	Project personnel involved in procurement should disclose relevant conflicts and avoid participating in decisions where impartiality is compromised.
Record retention	Maintain procurement and expenditure records in an organised project file for monitoring, reporting and donor verification.
Donor compliance	Apply any additional WA Foundation procurement, financial or reporting requirements communicated during contracting or implementation.

Table 10: Budget Controls



Figure 2: Approval Flow

10 PROJECT SUSTAINABILITY

FCHR is designed as a catalytic rather than dependency-based intervention. Grant resources will strengthen community-facing systems, tools, outreach and referral capacity that can continue to generate health and institutional value after the funding period.

Sustainability Pathway	How it will support continuity
Insurance and health linkages	Expand pathways that enable eligible community members to access care through community insurance relationships.
Community health financing	Develop affordable community/membership approaches where appropriate and feasible.
Service-revenue pathways	Increase appropriate use of diagnostics, pharmacy and clinical services arising from better referral and continuity.
Strategic partnerships	Develop government, private-sector, development-partner and community relationships that can carry forward activities.
Social enterprise and training	Explore health-promotion and capacity-building opportunities that can generate modest programme income while retaining social impact.
Institutional investment and philanthropy	Continue FairBanks resource mobilisation for priority community programmes after the grant period.

Table 11: Sustainability Pathway

10.1 Post-Grant Continuity Plan

By Month 12, FairBanks will maintain the community map, VHT/CHW relationships, referral tools, follow-up processes and FCHIP reporting structure established through the programme. The endline review will identify the most effective activities and the resources required to continue or scale them. Sustainability decisions will be based on community demand, documented outcomes, partnership commitments and available financing pathways through the following ways:

- Maintain the community stakeholder and referral network.
- Integrate effective community engagement activities into FairBanks routine outreach planning.
- Use programme evidence to support future grant, partnership and co-financing applications.
- Prioritise activities for continuation according to demand, documented results and available resources.

11 PROJECT PERSONNEL AND STAFF MANAGEMENT PLAN

The project will use a management structure that assigns clear accountability while leveraging on existing FairBanks capacity.

Name / Staff	Qualification	Skills	Core Responsibility
Racheal Nabukeera – Managing Director / Project Lead	BA Social Sciences – Makerere University; Master’s in Social Sector Planning & Management –	Strategic leadership; healthcare management; project coordination; stakeholder engagement;	Overall project leadership; strategic planning; stakeholder and partner coordination; resource mobilisation; implementation

FairBanks Community Health Reach Project

| WA Foundation Category 2 |

	Makerere University; PhD Scholar in Management – Uganda Christian University	resource mobilisation; partnership development; HR and organisational management	oversight; accountability and reporting
Dr. Twalib Olega Aliku – Clinical Lead	MBChB – Makerere University; MMed Paediatrics & Child Health – Makerere University; Fellowship in Paediatric Cardiology – Uganda Heart Institute; Level II Master in Cardiac Surgery & Cardiology – International Heart School/University of Milan; MBA – ESAMI	Paediatric cardiology; cardiovascular screening; clinical leadership; specialist outreach; research; training; referral coordination; telemedicine; task-sharing	Lead clinical implementation; oversee clinical protocols and quality of care; coordinate cardiovascular screening, referral and follow-up; supervise clinical teams and specialist outreach
Dr. Godfrey Alia – Senior Clinical / Maternal & Reproductive Health Lead	MBChB – Makerere University; MMed Obstetrics & Gynaecology – Makerere University; Diploma in Reproductive Health – Karolinska Institute; Fellow ECSACOG; specialist training in genital fistula surgery	Obstetrics & Gynaecology; reproductive health; maternal health; urogynaecology; specialist surgery; outreach; clinical governance; training; reporting	Provide specialist oversight for maternal and reproductive health; support outreach, referrals and specialist services; strengthen clinical governance and mentorship
Dr. Tendo James Lubwama – Medical / Outreach Officer	MBChB – Kampala International University	Patient care; diagnostics; emergency medicine; POCUS; obstetric and paediatric emergencies; health education; community outreach; research/data handling; project management	Support clinical service delivery, community screening and outreach; provide assessment and referral; support emergency response, health education, data collection and continuity of care
Frank Sekagiri – Sociologist / Community Development Lead	MA Sociology – Makerere University; Postgraduate Diploma in Public Administration & Management – UMI; BA Social Sciences – Makerere University; Certificate in Project Planning & Management; Certificate in Community Mobilisation	Community development; sociology; social policy; stakeholder engagement; participatory planning; M&E; gender mainstreaming; social protection; advocacy; capacity building	Lead the social/community development strategy; stakeholder mapping and engagement; community mobilisation; social inclusion and gender considerations; participatory planning; social safeguards and community feedback
Diana Kawoozo – Community Mobilisation Lead/ Coordinator	1st Class MBA – ESAMI; 1st Class PG Diploma HRM – UMI; 1st Class BA Social Sciences – Makerere University; Certificate in Law – LDC	Stakeholder engagement; strategic communication; public speaking; administration; quality management; report writing; teamwork; coordination	Support community coordination, stakeholder communication, administration, documentation and implementation planning
Kigenyi Conrad – Community Relations / Field Mobiliser	Diploma in Clinical Medicine and Community Health – Mbale College of Health Sciences	Community mobilisation; health education; fieldwork; referrals; client relations; marketing; outreach coordination; customer care; follow-up	Coordinate field mobilisation; work with VHTs and local leaders; conduct health education; support referrals, special clinics and client follow-up
Niwaereza Janet – Community Engagement / Outreach Support	Bachelor’s Degree in Community Based Rehabilitation – Kyambogo University	Community engagement; mobilisation; community-based rehabilitation; administration; marketing; communication; coordination; report writing; documentation	Support community mobilisation, awareness and outreach activities; beneficiary engagement; referrals; community feedback and outreach reporting
Wasswa Wilson – M&E / Data Lead	BSc Biomedical Engineering (Second Upper	M&E; health information systems; data quality;	Lead M&E and data systems; develop and maintain FCHIP;

	Honours) – Makerere University; Certificate in Leadership & Management in Health – University of Washington; Java Programming Level 1	database development; dashboards; data analysis; digital systems; reporting; data security	oversee data capture, quality assurance, analysis, dashboards and project reporting
Joseph Karamagi – Project Finance Officer	MBA (Accounting) – Makerere University; BSc Economics & Statistics – Kyambogo University; Postgraduate Diploma in Tax & Revenue Administration – East African School of Taxation; CPA (ICPAU) studies	Financial management; audit; taxation; budgeting; project costing; economic & financial analysis; accounting systems; financial advisory; compliance	Provide overall financial leadership and oversight; budget and expenditure management; financial controls; compliance; financial reporting; audit readiness; financial sustainability and risk
Tonny Gingo Wilson – Project Accountant	Bachelor’s Degree in Business Administration (Accounting) – Makerere University Business School; Level 2 Accounting, Finance & Taxation – ICPAU; CISCO ICT & Software	Accounting; reconciliations; grant accounting; accounts payable/receivable; budget tracking; donor compliance; tax compliance; audit support	Day-to-day project accounting; transaction processing; reconciliations; budget-versus-actual tracking; grant financial records; financial reporting and audit documentation
Tyra Rebecca Nalukwago – Procurement Manager	BSc (Hons) Land Economics – Makerere University; Graduate Member, Institute of Surveyors of Uganda	Procurement coordination; field planning; stakeholder liaison; documentation; data management; market research; client relations; GIS; Microsoft Office	Lead procurement planning and documentation; coordinate suppliers and procurement activities; maintain procurement records; support transparency, compliance and value for money
VHTs / CHWs / Community Actors	Relevant VHT/CHW training and community-health experience	Community mobilisation; health education; beneficiary identification; referrals; follow-up; local communication; community sensitisation	Mobilise beneficiaries; support sensitisation and screening; facilitate referrals and follow-up; provide community feedback and support community-level data collection

Table 12: Staff Management Plan

12 PARTNERS

Partnerships are a core mechanism of FCHR. Community engagement partners specifically identified in the existing FairBanks engagement framework include LCI and II chairpersons, VHTs, CHWs, youth leaders, women’s group heads and religious leaders. Their intended roles include mobilisation, sensitisation, household identification, community awareness, referral support and community feedback.

Partner Type	Proposed Role in FCHR	Status / Verification Requirement
community structures	Community entry, mobilisation, local coordination and feedback.	Central A; Erisa; Kalerwe; Quarters; Katale; Kitabuliiki; Nsooba and Tuula. Shilo School; Cleveland Hill Primary School; Passover Church; St Pauls Church Kyebando; Jericho Church
Local leaders		Kagimu Lazaru Kanyike - Chair Man (Central) Mayinja Amuli - Chair Man (Erisa)

		Kimbugwe Joseph - Chair Man (Katale) Makubuya Stephen - Chair Man (Bukoto) Kakooza Joseph Sande - Chair Man (Mulimira)
VHTs / CHWs	Household/community mobilisation, education, screening support, referral and follow-up.	Agoroi George Vincent Aisha Kizza Mugisha Ronald Nalumansi Gladys Nakisozi Masitulah Makooma Muzamiru Tibakunurwa Vicky Nalwanga Sarah Nalongo Nanyanzi Stella Kissarach Alvin Nansubuga Asiah
Health / clinical referral partners	Specialist or higher-level referral where care is outside FairBanks scope.	Mulago Referral Hospital; Bukoto Health Centre III; Komamboga Health Centre III; KCCA Kamwoya Community Centre; Nakasero Blood Bank
Insurance / financing partners	Coverage, referral and community financing pathways.	CIC; AAR; Sanlam Allianz and Centenary Bank
Development / private-sector partners	Technical support, commodities, outreach support or co-financing where agreed.	Ministry of Health Uganda; Uganda Heart Institute; Micro Labs Ltd/ Micro Labs Pharmacy; Denk Pharma; Kawempe Division Health Offices; Aiveen Uganda Limited;

Table 13: Partners matrix

13 PROJECT ACCOUNTABILITY STRUCTURE

Accountability will operate at project governance, strategic governance, project management, financial, clinical, Data Management, community and funder levels. Project and financial records will be maintained against approved activities and budget lines.

Level	Responsible Function	Accountability Mechanism
Project governance	Project Management Committee	Budget oversight, Risk management, Quality assurance, Performance monitoring Compliance and accountability, Approval of plans, Monitoring project sustainability, Project closure
Strategic governance	Managing Director and designated senior leadership	funder communication, facility oversight, Stakeholder coordination, Problem-solving and decision-making, Change control, Reporting
Project management	Project Lead with technical teams	Monthly activity review, workplan tracking, issue escalation and routine reporting.
Financial accountability	Finance/Administration function	Budget implementation, supporting documentation verification, reconciliations and financial reporting.
Clinical quality	Clinical Lead / designated clinician	Clinical protocols, referral quality, patient safety and service linkage oversight.
Data Management	IT Technician	Data Capture, Data Processing, Cleaning and retrieval
Community accountability	Community mobilisation lead with VHT/CHW and local structures	Community feedback, participation records, complaints/concerns escalation and response tracking.
Funder accountability	Project Lead + Finance/Administration + Executive Oversight	Narrative and financial reports, evidence files and readiness for verification.

Table 14: Accountability Mechanism Matrix

14 MONITORING, EVALUATION AND LEARNING PLAN

Monitoring will combine routine programme data capture, referral tracking, community feedback, quarterly review and an endline assessment. Baseline work will validate the operational targets proposed in the full proposal and establish the final beneficiary denominator for the selected communities.

Output	Indicator	Frequency	Primary Source
Community Reach	Number of community members/households reached	Monthly	Mobilisation/outreach records
Health Education	Number receiving structured health education	Monthly	Session registers
Screening	Number screened and number identified at risk	Monthly	Screening records
Referral	Number referred and proportion completing referral	Monthly/Quarterly	Referral register and facility linkage records
Continuity	Number/proportion receiving documented follow-up	Monthly	Follow-up records

Equity	Participation of priority vulnerable groups	Quarterly	Disaggregated participation records where feasible
Quality	Community/client feedback and satisfaction	Quarterly	Feedback tools
Learning	Programme review, lessons and adaptation actions	Quarterly	Review minutes and learning notes

Table 15: Monitoring Plan

14.1 Proposed Indicator Matrix

Indicator	Target	Timing	Evidence Source
Communities mapped and engaged	At least 08 priority communities	Inception / updated quarterly	Community map and engagement records
VHTs/CHWs oriented	At least 30 community health actors	Months 1-2; refreshers as needed	Orientation attendance and support records
People reached through health education/mobilisation	At least 6,000	Monthly	Session and mobilisation registers
People screened	At least 2,000	Monthly	Screening registers
Individuals needing further assessment/referral	At least 300	Monthly	Screening and referral records
Referrals initiated and tracked	At least 300	Monthly	Referral register
Referral completion	At least 70% of trackable referrals	Monthly/Quarterly	Referral and facility linkage records
Follow-up contacts	At least 500 documented contacts	Monthly	Follow-up register
Structured education sessions	At least 48	Monthly	Session registers
Programme reviews	Quarterly plus endline	Quarterly / endline	Review minutes and endline report

Table 16: Proposed Indicator Matrix

14.2 Learning and Adaptive Management

Quarterly reviews will compare implementation progress with targets, identify bottlenecks in mobilisation, screening, referral and follow-up, and document decisions to adapt implementation. The endline assessment will summarise outcomes, lessons and scale-up recommendations.

14.3 Key Risks and Mitigation Measures

Risk	Mitigation Measure
Community participation lower than expected	Early engagement of local leaders/VHTs; phased mobilisation; adapt session timing and delivery channels.
Referral completion lower than expected	Appointment linkage; reminder/follow-up; review reasons for non-completion; strengthen clinical navigation.

Data quality or incomplete records	Simple standardised tools; orientation; routine data checks; supervision and quarterly audits.
Insufficient resources for non-covered clinical needs	Clear referral criteria; partnership mapping; prioritisation of project-linked inputs; sustainability/financing pathways.
Delays in procurement or programme start	Front-load planning and quotations; phase procurement; maintain a clear implementation calendar.
Funding or partnership uncertainty after grant period	Begin sustainability and partnership development by mid-year; document continuation pathways before Month 12.

Table 17: Mitigation Measures

15 CONCLUSION

FairBanks Community Health Reach offers a practical bridge between the underserved peri-urban communities in Kampala and formal healthcare services. By combining community participation, prevention, screening, referral, clinical linkage, follow-up, health intelligence and sustainability mechanisms, the project is designed to create measurable improvements in access and continuity while leaving behind stronger community and institutional systems.

FairBanks Medical Centre Limited respectfully submits this full proposal to Within Africa Foundation for consideration. The facility is prepared to provide supporting documentation, detailed procurement information, verification materials and any further technical clarification required during the Category 2 review process.

16 ANNEXES FOR CATEGORY 2 SUBMISSION

Sn	Document / Evidence	Purpose	Submission Status
1	Certificate of registration / incorporation	Confirms legal identity and registration status	Annex 1
2	Relevant statutory / regulatory registrations and licences	Supports institutional and facility legitimacy	Annex 2
3	Fairbanks’ Organogram	Provides the facility’s governance, functional structure and staff reporting relationships	Annex 3
4	Project organogram	Shows project governance and reporting lines	Annex 4
5	Financial controls / accounting procedures	Supports grant accountability and financial management	Annex 5
6	Evidence of community engagement / partnerships / referrals	Demonstrates relevant operational relationships	Annex 6

Annex 1: Certificate of registration / incorporation

CERTIFIED TRUE COPY
CERTIFIED ON THU, 15 MAR 2023 11:54:27
VC20031514543786

Certificate issued on: 04-10-2022 12:34, No: BRS-INCC-9-22/13825

Registration No: 80020003843337


THE REPUBLIC OF UGANDA
THE COMPANIES ACT

Certificate of Incorporation

(Under section 18(3) of the Companies Act 2012)

I CERTIFY that FAIRBANKS MEDICAL CENTRE LIMITED (Limited by Shares) has this day been incorporated with Limited Liability.

Dated at Kampala, this 4th day of October the year 2022.

UGANDA REGISTRATION
SERVICES BUREAU


Signature: SOLOMON MULIISA
.....
Registrar of Companies



Printed on 12:43:05 04-10-2022

Annex 2: Relevant statutory / regulatory registrations and licences

THE REPUBLIC OF UGANDA
THE MEDICAL AND DENTAL PRACTITIONERS ACT, 1998

Registration Certificate / Operational Licence of Health Unit Premises
This is to Certify that

55992 Certificate Serial No.
31827 Receipt No.

NABUKERA RACHIAL hereinafter, referred to as the
owner is hereby authorised to register and operate a health Unit known as **FAIRBANKS MEDICAL CENTRE**
Specialised Establishment located at **KYEBANDO KUSALOSALO** in the
KAWEMPE subcounty of **KAMPALA** District.
The health Unit will be supervised by Dr **KUSASIRA SHAKA GHOFFER** Qualifications **MEDS(BEL)ING/2015**

Licensed for the calendar year **2026**

Fees: **500,000**
Date of Issue: **02 FEBRUARY 2026**
Date of Expiry of license: **31 DECEMBER 2026**

OFFICE OF THE REGISTRAR
UGANDA MEDICAL AND DENTAL
PRACTITIONERS COUNCIL
P.O BOX 16115,
KAMPALA, UGANDA
TEL/FAX: 256-200-904427
EMAIL: registrar@umdp.org.ug
www.umdpc.org.ug

DR. CHARLES TUSHIME
Registrar

This licence must be visibly displayed in the registered premises

42907

NATIONAL DRUG AUTHORITY

License to Operate Retail Pharmacy

Issued under Section 14 of the Act and Regulation 11 of the National Drug Policy and Authority (Licensing) Regulations 2014

This is to certify that the business trading under the name of
FAIRBANKS MEDICAL CENTRE LIMITED
is licensed to operate a Retail Pharmacy

at the Physical Address: **INSIDE FAIRBANKS MEDICAL CENTRE,
TIRUPATI ROAD, KYEBANDO, KAWENPE
DIVISION**

TIN: **1053370026**

with Supervising Pharmacist
having Registration No **2305**

Premise No :NDA/PRE/RTP/0454

Valid up to :31 December, 2027

NATIONAL DRUG AUTHORITY
KAMPALA EXTRA REGIONAL OFFICE
24 JUN 2025
FOR THE AUTHORITY

This License must be prominently displayed in the premises to which it refers

42906

NATIONAL DRUG AUTHORITY

General Certificate of Suitability of Premises for Retail Pharmacy

Issued under Section 17 of the Act and Regulation 4(2) of the National Drug Policy and Authority (Certificate of Suitability of Premises) Regulations 2014

This is to certify that the premises named
FAIRBANKS MEDICAL CENTRE LIMITED
at the Physical Address: **INSIDE FAIRBANKS MEDICAL CENTRE,
TIRUPATI ROAD, KYEBANDO,
KAWENPE DIVISION**

TIN: **1053370026**

with Supervising Pharmacist
having Registration No **2305**

are considered suitable for carrying on the business of
Retail Pharmacy

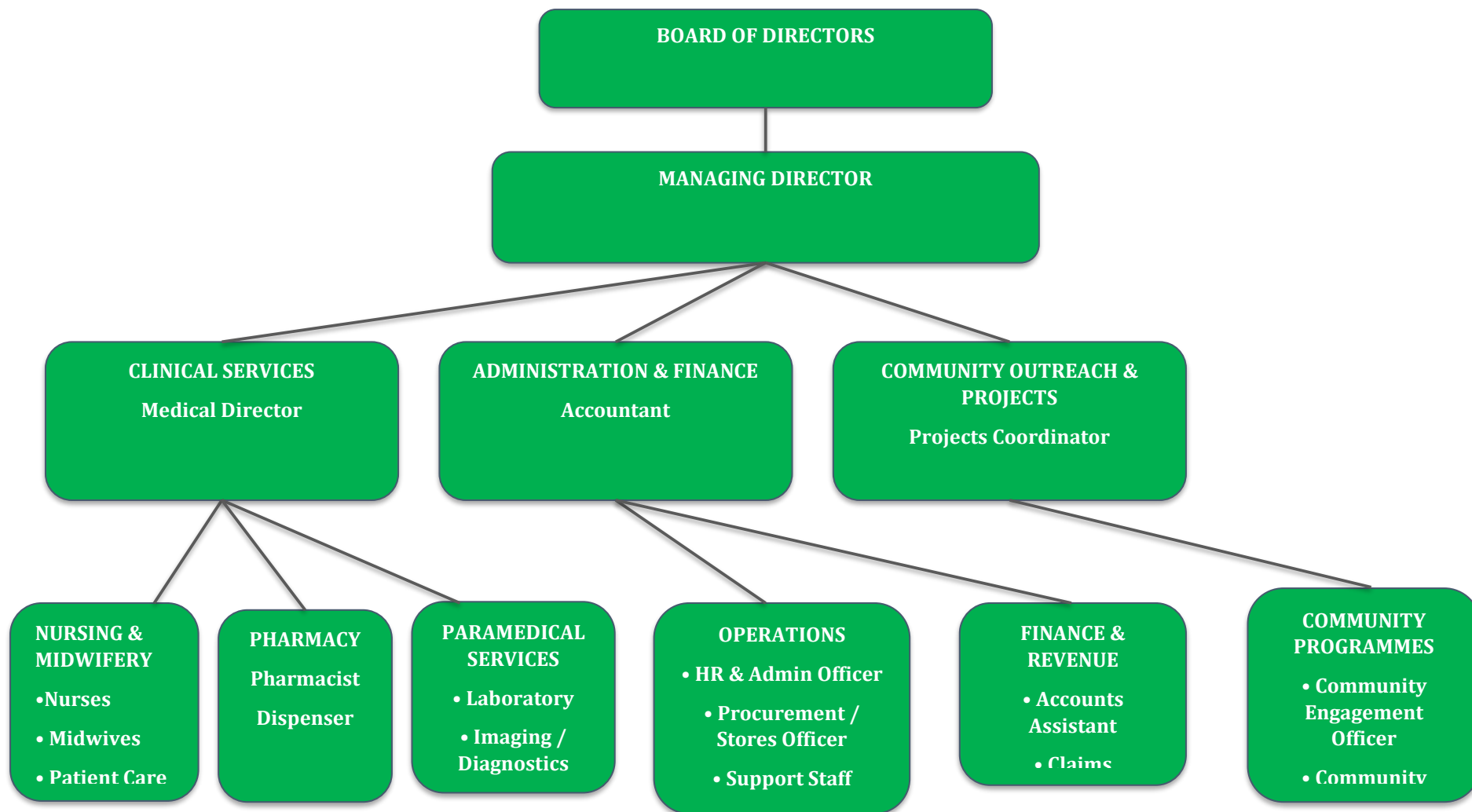
Premise No.NDA/PRE/RTP/0454

Valid up to :31 December, 2027

NATIONAL DRUG AUTHORITY
KAMPALA EXTRA REGIONAL OFFICE
24 JUN 2025
FOR THE AUTHORITY

This certificate must be prominently displayed in the premises to which it refers

Annex 3: Fairbanks Organogram

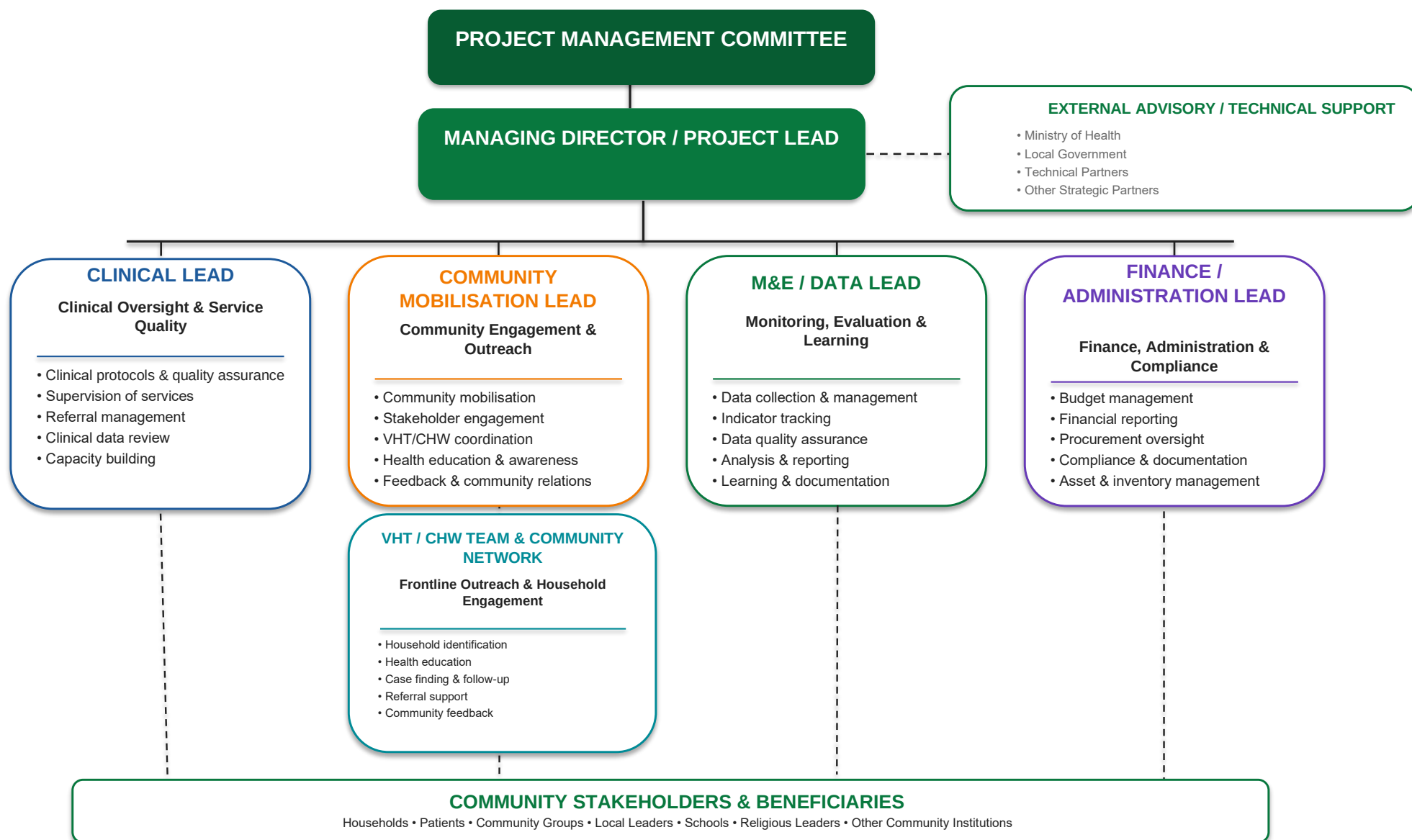


Annex 4: Project organogram

ANNEX 4 – PROJECT ORGANOGRAM

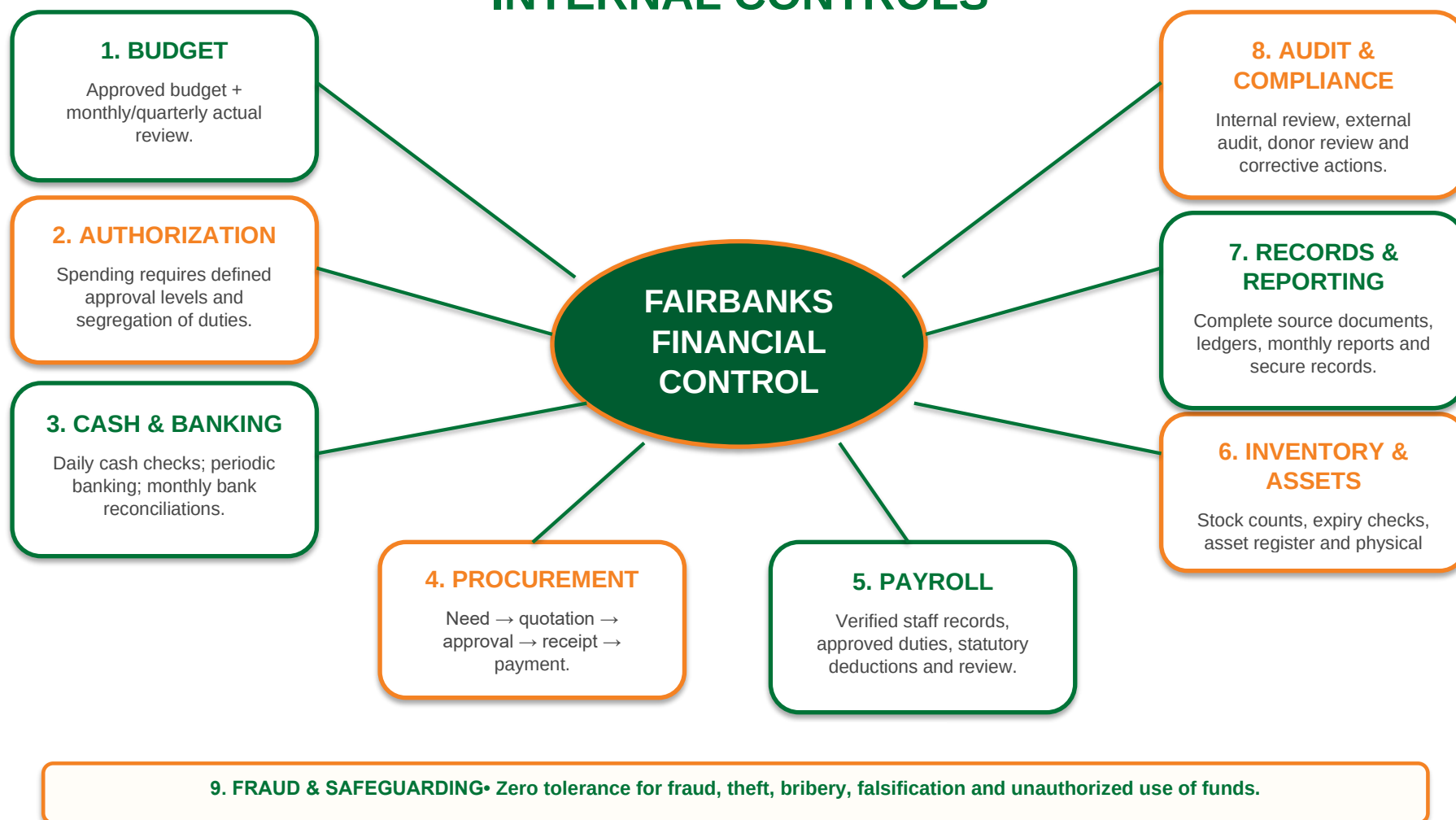
FAIRBANKS COMMUNITY HEALTH REACH

WA FOUNDATION CATEGORY 2 PROJECT



Annex 5: Financial controls / accounting procedures

ANNEX 5: FINANCIAL MANAGEMENT & INTERNAL CONTROLS



Annex 6: Evidence of community engagement / partnerships / referrals



Cardiology camp in progress



FBMR community engagement



Training of VHTS, Local leaders and CHWs at Fairbanks Medical Center

FairBanks Community Health Reach Project

| WA Foundation Category 2 |



The Geri Care Project



Blood donation during the Cardiology camp



Ophthalmologist engaging a client during an ophthalmology camp



Mothers carrying their babies on a Child Immunisation day

HOSPITAL/CLINIC KYC FORM

A. HOSPITAL DETAILS:

- Business Name: FAIRBANKS MEDICAL CENTRE LIMITED
- Postal Address: P.O. Box 11882 Kampala, Uganda
- Physical Address: Kyebanda - Kibukala
- Facility Telephone: 0748219052, 0777460398
- Facility Email: info@fairbanksmedicalcentre.ug
- Tax Identification Number (TIN): 1063370026
- Operational License No.: A6478
- Bank Account Name: FAIRBANKS MEDICAL CENTRE
- Bank Account No.: 3100118372
- Bank Name: Centenary Rural Development Bank
- Swift Code: CERBUKXXXX Branch: Majura

B. KEY CONTACT PERSON DETAILS:

- Director Full Name: DR. KINGOMA JOSEPH
- Telephone: 0734452537 / 0792320020
- Email: info@fairbanksmedicalcentre.ug
- Insurance Contact Person:
- Telephone:
- Email:

16. Force Majeure
Neither the Healthcare Provider nor the Company shall be responsible for any injury, damage or loss arising out of or in connection to any circumstances or occurrences beyond its control amongst others, fire, storm, flood, earthquake or other acts of God, epidemics, wars, strikes, with stoppages or slowdowns, epidemic or quarantine restrictions.

17. Modifications
Any alteration, waiver or modification of the terms of this Agreement shall not be valid unless signed by the Director of the Healthcare Provider and by a person authorized by the Company.

18. Governing law
The validity of this Agreement, its interpretation, enforcement, the respective rights and obligations of the parties and all other matters arising in any way out of it, shall be governed by and construed in accordance with the laws of Uganda.

In witness whereof the parties herein subscribe to this Agreement on the day and year first above written.

For and on behalf FAIRBANKS MEDICAL CENTRE Official Signatures

Name: Rachael Nabukera

Position: Managing Director

Date: 12th September 2025 (After Healthcare provider's KYC SEP 2025)

Name: DR. KINGOMA JOSEPH

Position: Medical Director

Date: 12th September 2025

For and on behalf of the CIC GENERAL INSURANCE UGANDA LTD

Name: Joselyn Amuge

Position: AG. CEO

Date: 22/09/2025 (After Healthcare provider's KYC SEP 2025)

In the presence of

Name: AFULWA KIGOROGA

Position: MEDICAL CLAIMS MANAGER

Date: 10th/09/2025

MEDICAL SERVICES AGREEMENT

Corporate clients

This Agreement is made on 12 day of September (Month) and 2025 (Year)

between

FairBanks Medical Centre (Hospital/Clinic P.O. Box 111882)

(Hereinafter referred to as "the Healthcare Provider") which expression shall where the context so admits include its successors and assigns) of one part, and

CIC GENERAL INSURANCE UGANDA LIMITED of P.O. Box 34875 Kampala, Uganda, (hereinafter referred to as "the Company/CIC") which expression shall where the context so admits include its successors and assigns) of other part.

The Healthcare Provider and the Company shall hereinafter be individually referred to as a "Party" or collectively as "Parties".

Whereas

- The Healthcare Provider is an accredited health care provider licensed to provide quality medical services and treatments;
- The Company is a licensed provider of medical insurance services under which it operates healthcare scheme(s) for eligible members;
- The Company wishes to engage the Healthcare Provider to provide such Health care services (hereinafter referred to as "the services") to eligible members under the healthcare schemes insured;
- The Healthcare Provider has represented to the Company that it has the required competent professional personnel with competence and is duly equipped with relevant technical resources necessary for the provision of the required healthcare services relating to this Agreement;
- The Healthcare Provider is willing and capable of providing the said healthcare services and has for this purpose agreed to open an account for the Company, subject to the terms and conditions of this Agreement to provide services within its scope, capabilities and resources.

1. Definitions and Interpretations

In this Agreement the following words will have the meanings assigned to them in this clause, except where inconsistent with the context.

"Agreement" means this Agreement including any schedules or annexures and as varied from time to time;

"Business Day" means any Calendar day apart from Saturday, Sunday or a national holiday in Uganda;

"Commencement Date" means the date indicated at the top of this page or any other date as the Parties may agree in writing;

"Data" means any information about CIC General Insurance Uganda Limited employees or individuals and/or any CIC client information which might be processed by the Healthcare Provider as part of the Services.



Kyebanda - Kibukala
P.O. Box 111882, Kampala - Uganda


 THE REPUBLIC OF UGANDA
 AGREEMENT FOR PROVISION OF MEDICAL
 EXAMINATIONS SERVICES FOR FOOD HANDLERS IN
 DIVISION A (LOT 2)
 PROCUREMENT REFERENCE NUMBER
 ENTE 705/SRVCS/2025-2026/00013
 PROVIDER
 M/S FAIR BANKS MEDICAL CENTRE
 CLIENT
 ENTEBBE MUNICIPAL COUNCIL
 DATE:
 SEPTEMBER, 2025

Our Vision: To be A Model Self-sustaining Municipality with Prosperous People by 2040
www.entebbe.go.ug

IN WITNESS WHEREOF, the parties have caused this Contract to be signed in their respective names as of the day and year first above written.

1. Signed: [Signature] For Entebbe Municipal Council
 Name: MUGISHA EMMANUEL Position: TOWN CLERK
 Date: 15 SEP 2025

2. Signed: [Signature] For Entebbe Municipal Council
 Name: [Signature] Position: Principal Treasurer

3. Signed: [Signature] For Entebbe Municipal Council
 Name: [Signature] Position: Municipal Health Officer

4. WITNESS
 Signed: [Signature] For Entebbe Municipal Council
 Name: [Signature] Position: Head, Procurement and Disposal Unit

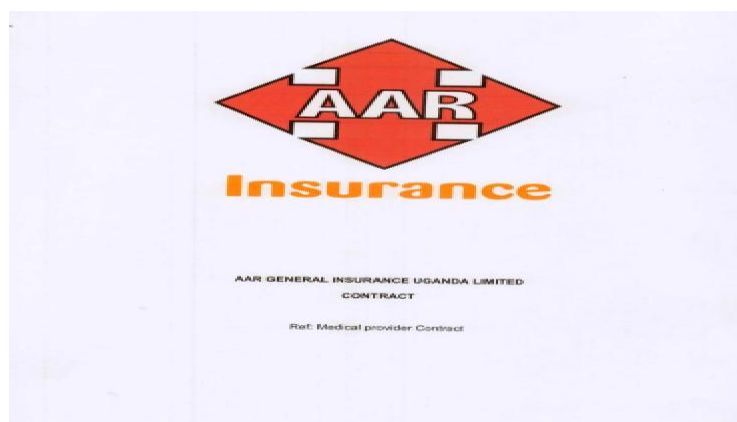
FOR THE SERVICE PROVIDER:

1. Signed: [Signature] Position: Authorized Representative
 Name: SEKATUNZI FREDRICK Fair Banks Medical Centre

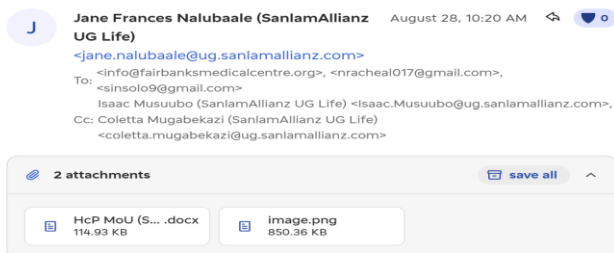
WITNESS

2. Signed: [Signature]
 Name: [Signature]





Onboarding to SanlamAllianz Provider Care – MOU & Required Documents



Hi Fair banks Medical,

Further to your request to be added onto the SanlamAllianz Provider Care network, thank you for your interest in joining us. We truly appreciate it.

Please find attached our Memorandum of Understanding (MOU) for your review.

To proceed with onboarding, kindly share the following documents at your earliest convenience:

1. Operating License
2. Certificate of Incorporation
3. Memorandum and Articles of Association
4. Tax Registration Certificate
5. Price List
6. Beneficial Owners' Information

Kind Regards,

Kind Regards,

Jane Frances Nalubaale

Provider Network Officer

Sanlam Allianz Life Insurance Uganda Limited

Landline: +256 323 526 526 **Cell Phone:** +256 785 135 568

Plot 15 Princess Anne Drive Bugolobi, Kampala | PO Box 25495, Kampala, Uganda.



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Please note that the on boarding process is still in progress